

THE 100-DAY DIABETES EDUCATION CONTENT PLAN

A Pharm.D + MSc Diabetes Care & Management Creator's Playbook

Instagram Reels (1-2 min) + YouTube Long-Form (5-6 min)

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PART A -- DAYS 1 TO 14 -- FULL WORD-FOR-WORD SCRIPTS

How to read each day: Pick one hook from the three options based on the day's mood. The Instagram script is timed in seconds. The YouTube script is timed in minutes:seconds. References at the bottom of each day go in your video description and pinned comment.

DAY 1 -- Welcome: Why this channel exists

Hook lines (pick one)

If I'm uncertain, I'll say I'm uncertain."

[1:30-2:30] What "evidence" means here "Evidence has a hierarchy. At the bottom: my opinion, your friend's opinion, a TV anchor's opinion. In the middle: case reports -- one patient who got better. Higher up: cohort studies -- many patients followed over time. Higher still: randomised controlled trials, where people are split by chance and one group gets the treatment. At the top: systematic reviews and meta-analyses, which combine many trials. When I say 'studies show,' I'll tell you which level of evidence I'm pulling from."

[2:30-3:30] What this 100-day series covers "For the next 100 days, I'll cover ten themes. Foundations. Pathophysiology. Diet, especially the Indian thali. Exercise. Yoga, sleep and stress. Medications, drug class by drug class. Reversal and remission, honestly. Complications and how to prevent them. Special groups -- pregnancy, children, elderly, Ramadan. And finally, misinformation -- how to read a testimonial, how to read a label, and what to do when an influencer contradicts your doctor."

[3:30-4:15] What I will not do "I will not sell you a course in this series. I will not promise you can be off insulin in 72 hours. I will not tell you Type 1 diabetes is reversible -- because it isn't, and saying that costs lives. I will not name and shame other creators; I will only debunk ideas. And I will always, always, tell you to stay in touch with your own doctor."

[4:15-5:00] What you should do "If you have diabetes, save this video. Share it with the family member who cooks your food and the one who does grocery shopping -- they need this more than you. If you don't have diabetes but have a parent who does, this is your homework now. Subscribe, hit the bell. Tomorrow, in under five minutes, I'll explain what diabetes actually is -- without the jargon."

[5:00-5:30] Sign-off "This is your starter pack. The disclaimer is pinned: this is education, not personal medical advice. Talk to your physician before changing anything. See you tomorrow. Day 1 of 100."

References

Hook lines

blurred vision -- the damage to small blood vessels has already started.¹? This is why screening matters. This is why treating the number matters. Tomorrow we'll go deeper into the glucose-insulin handshake. See you then."

References

[0:20-1:30] What insulin really does "Insulin is a hormone made by the beta cells of the pancreas. Think of it as a postman delivering three messages. Message one, to muscle and fat cells: open your doors, glucose is here. Message two, to the liver: stop pouring out glucose, the meal has arrived. Message three, to the whole body: store the surplus -- as glycogen in liver and muscle, and as fat in adipose tissue, for the next fasting period.¹¹ Without these three messages, glucose stays trapped in the blood, and the body literally starves in the middle of plenty."

[1:30-2:30] The fed state vs the fasting state "After a meal, insulin rises. Glucose falls. The body is in 'fed state' -- storing energy. Between meals or overnight, insulin falls and another hormone, glucagon, takes over. Glucagon tells the liver to release stored glucose. This back and forth is how your blood sugar stays in the 70-to-140 mg/dL range no matter what you eat. In diabetes, the back and forth breaks. Either insulin doesn't come (Type 1), or it comes but the cells ignore it (Type 2)."¹²

[2:30-3:30] Why the 'insulin makes you fat' line is misleading "Yes, insulin promotes storage. But insulin only acts on what you eat. If you eat 1,500 calories, insulin manages 1,500 calories. The reason people gain weight in modern life is excess calories -- especially refined carbohydrates and ultra-processed snacks -- not insulin existing. Studies that compare low-carb to low-fat diets, when calories are matched, show very similar weight loss.¹³ The honest summary: insulin is a tool. Cutting refined carbs lowers insulin spikes, which is helpful for many T2D patients. But demonising the hormone itself is wrong."

[3:30-4:30] What happens without insulin "Imagine the postman is gone. Glucose arrives in the blood after a meal but cannot enter cells. The blood sugar climbs to 300, 400, 500 mg/dL. The cells, starving, send signals to the liver to make more glucose -- which makes the blood sugar even higher. The body shifts to burning fat for fuel, producing ketones, which acidify the blood. This is diabetic ketoacidosis: nausea, vomiting, deep rapid breathing, confusion, coma. Without insulin, a Type 1 diabetic can be in DKA in less than 24 hours.¹⁴ This is why nobody -- no influencer, no diet, no oil -- can replace insulin in Type 1."

[4:30-5:30] What it means for you "If you are on insulin and someone tells you to stop because insulin is bad -- please ignore them, then call your doctor before you do anything. If you are on metformin or other tablets, none of those tablets are insulin; they help your own pancreas or your own cells use insulin better. Tomorrow we'll go inside the pancreas itself. See you in the next one."

References

DAY 4 -- The pancreas: your sugar factory

Hook lines

[3:15-4:15] How alpha cells decide "Alpha cells do the opposite. When blood glucose falls -- between meals, during exercise, while you sleep -- alpha cells secrete glucagon. Glucagon travels to the liver and tells it to break down glycogen, the body's stored sugar, and release glucose into the blood. In Type 2 diabetes, there is a strange and underappreciated problem: alpha cells become inappropriately active even after meals, secreting glucagon when they shouldn't. The liver dumps glucose right when you've just eaten. This is one of the eight defects in DeFronzo's 'Ominous Octet' theory of T2D.¹?"

[4:15-5:00] What this means in practice "This anatomy lecture has a practical point. It tells you why we use multiple drugs -- because Type 2 diabetes isn't one defect. Metformin works on the liver and muscle. SGLT2 inhibitors work on the kidney. GLP-1 receptor agonists work on the alpha and beta cells, the brain and the gut. Insulin replaces what the beta cell can no longer make. There is no single magic pill -- because there is no single broken part. Tomorrow, we'll talk about HbA1c -- your blood's three-month report card. See you in the next one."

References

YouTube (5 min 30 sec)

[0:00-0:20] Cold open "You can fast for 12 hours, drink only water, and walk in for a blood test that says your sugar is fine. And still -- you can be diabetic. The single test that catches you is HbA1c. Today we'll explain it like adults."

[0:20-1:30] What HbA1c actually measures "Inside every red blood cell is haemoglobin -- the protein that carries oxygen. Glucose floating in your bloodstream slowly attaches itself to haemoglobin in a process called glycation. The longer glucose has been high, and the higher it has been, the more haemoglobin gets glycated. We measure the percentage of haemoglobin that is glycated -- that's HbA1c. Because red blood cells live about 120 days, HbA1c reflects average glucose over the past 2 to 3 months.^{1?}"

[1:30-2:30] Reading the number "The American Diabetes Association cut-offs are: below 5.7% is normal, 5.7 to 6.4% is prediabetes, 6.5% or higher on two separate tests confirms diabetes.^{2?} The ICMR uses the same cut-offs in India. There's also an estimated average glucose -- eAG -- that translates HbA1c into mg/dL: 6% ? 126 mg/dL, 7% ? 154, 8% ? 183, 9% ? 212, 10% ? 240. So when your doctor says 'HbA1c of 9 is bad', what she means is your average sugar for three months has been over 200."

[2:30-3:30] Why HbA1c matters more than a single fasting sugar "Fasting glucose is one snapshot. After you eat, your sugar can spike to 250 and come back down -- and morning fasting will look fine. HbA1c sees those spikes. The UKPDS trial, which followed thousands of T2D patients for over a decade, showed that every 1 percentage-point reduction in HbA1c reduced microvascular complications -- eyes, kidneys, nerves -- by about 37%, and reduced heart attack risk by about 14%.²¹ So this number is not for your doctor's file. It is for your eyes, your kidneys, and your heart."

[3:30-4:30] When HbA1c lies "HbA1c is not perfect. Anything that changes the lifespan of red blood cells changes the test. If your red cells live shorter -- for example in iron-deficiency anaemia after treatment, in haemolytic anaemia, in pregnancy, in chronic kidney disease, after blood loss -- your HbA1c will read falsely low. If your red cells live longer -- untreated iron deficiency, vitamin B12 deficiency, alcoholism -- your HbA1c can read falsely high.²² India also has a high prevalence of haemoglobin variants like HbE, HbS and beta-thalassaemia trait, especially in eastern and southern states; these can interfere with certain HbA1c assays.²³ This is why we don't diagnose diabetes on HbA1c alone in such patients -- we add a fasting sugar or an OGTT."

[4:30-5:30] How to use this number "If your HbA1c is in prediabetes range, you have a window. Lifestyle alone can prevent or delay diabetes by up to 58% over 3 years, as shown in the Diabetes Prevention Program.^{2?} If you are diabetic, the target is usually under 7%, but it depends on your age, complications, and risk of low sugar -- a 75-year-old with heart disease should not aim for 6%. Recheck every 3 months if you're adjusting treatment, every 6 months if stable. Tomorrow we'll go even deeper -- fasting, post-meal and random sugars, and what they actually tell your doctor. See you then."

References

B-roll

fine but the patient is high-risk."

[2:20-3:20] Random plasma glucose (RPG) "This is taken at any time of day, irrespective of meals. It's not used for routine diagnosis because of meal interference, but a single value of 200 mg/dL or higher with classic symptoms -- excess thirst, frequent urination, weight loss, blurred vision -- is sufficient for a diabetes diagnosis without retesting.²? This is the test we use in emergency settings."

[3:20-4:00] What about glucometer readings at home? "A finger-prick glucometer is great for trends, not for diagnosis. Lab plasma readings are about 10 to 12% higher than capillary readings on the same person at the same time, because the meter measures whole blood. Treat your meter as a self-monitoring tool, not as the source of diagnosis. For diagnosis, always go to a NABL-accredited lab.²?"

[4:00-5:00] Putting all three together "A complete picture of your glucose has three layers. Fasting tells me about your liver and overnight metabolism. Post-meal tells me about your beta cells' response to food. HbA1c -- yesterday's video -- gives me the average over 3 months. Each number can be misleading on its own. Together, they paint the truth. Tomorrow we'll talk about the symptoms most people ignore until it's too late. See you then."

References

YouTube (5 min)

[0:00-0:20] Cold open "By the time most Indians are diagnosed with diabetes, the disease has been running silently for 5 to 7 years.^{2?} The complications begin before the diagnosis. Today, learn what the body has been trying to tell you."

[0:20-1:30] The classical four "Polyuria -- frequent urination, especially at night. When blood sugar crosses about 180 mg/dL, the kidneys can no longer reabsorb all of it; glucose spills into urine and pulls water with it.^{2?} Polydipsia -- extreme thirst -- follows because of the fluid loss. Polyphagia -- excessive hunger -- happens because the cells, despite plenty of glucose in blood, are starving. And finally weight loss, especially in Type 1 or in late Type 2, because the body shifts to burning fat and muscle when it cannot use glucose. If you have all four, please get tested today, not next month."

[1:30-2:30] The silent symptoms most patients miss "Slow-healing wounds -- high blood sugar weakens immune cells and impairs blood vessel function, so a small cut takes weeks to close.^{3?} Recurrent skin and yeast infections -- fungi love sugar, so balanitis in men, vulvovaginitis in women, and skin folds infections may be the first sign. Tingling, numbness or burning in the feet -- early diabetic neuropathy. Blurred vision that fluctuates with meals -- sugar swelling the lens.³¹ A dark, velvety patch on the neck, armpits, knuckles -- acanthosis nigricans, which signals insulin resistance. Frequent gum infections and tooth loosening. Recurrent UTIs. Sudden erectile dysfunction in a previously healthy man."

[2:30-3:30] Symptoms by type "In Type 1, especially in children, symptoms appear over weeks, dramatically: a thin child suddenly drinking 5 litres a day, wetting the bed at age 8, losing 5 kg in a month. The next step is often diabetic ketoacidosis -- vomiting, deep rapid breathing, abdominal pain -- which is a medical emergency. In Type 2, symptoms are slow, mild and often dismissed for years. Many patients are diagnosed accidentally during a master health check or a pre-surgical work-up."

[3:30-4:15] Symptoms in women specifically "In women, especially Indian women, the early signs may be PCOS-like: irregular periods, acne, facial hair, weight gain around the abdomen. PCOS itself increases the risk of Type 2 diabetes 4-fold over a lifetime.³² A history of gestational diabetes raises the risk by 7-fold within 10 years.³³ If you are a woman with PCOS, GDM history, or a family history, please get a yearly HbA1c."

[4:15-5:00] When to see a doctor today "If you have any of these -- unintentional weight loss greater than 5% of body weight in 3 months, unexplained vomiting, deep rapid breathing, breath that smells like nail-polish-remover, persistent abdominal pain in someone with diabetes, fainting, confusion -- go to the emergency department. Do not wait for a clinic appointment. Tomorrow we'll talk about screening -- who should get tested, and how often. See you in the next one."

References

Disclaimer: Education only.
#DiabetesSymptoms #DiabetesIndia #PharmD

DAY 8 -- Who should get screened and when

Hook lines

prediabetes and diabetes is your one chance to turn back; don't waste it."

[4:15-5:00] What to do with the results "If your fasting sugar is 100 to 125, or HbA1c is 5.7% to 6.4%, you have prediabetes. Don't panic -- but don't ignore it. Lose 5 to 10% of your body weight, walk 150 minutes a week, cut refined carbs, sleep 7 hours, and re-test in 3 months. If your fasting is 126 or higher, or HbA1c is 6.5% or higher, see a doctor. Tomorrow we'll cover the OGTT -- the gold-standard test that catches what others miss. See you then."

References

minutes. A second blood sample is drawn at exactly 2 hours from the first sip.?? For pregnant women, a 100-gram, 3-hour version exists, with samples at 0, 1, 2 and 3 hours, and different cut-offs apply.?? For diagnosis of gestational diabetes, the WHO and DIPSI now recommend a single 75-g, 2-hour test."

[1:30-2:30] Reading the result "A 2-hour value below 140 mg/dL is normal. Between 140 and 199, you have impaired glucose tolerance -- a form of prediabetes. 200 or higher confirms diabetes -- though we typically need a second confirmatory test on a different day, unless symptoms are unmistakable.??

Note: the fasting sample also matters. Some people have normal fasting and abnormal 2-hour values -- isolated post-prandial hyperglycaemia. Others have abnormal fasting and normal 2-hour -- isolated fasting hyperglycaemia. Both are real prediabetes, with different mechanisms."

[2:30-3:30] Why it catches what other tests miss "In the Chennai Urban Rural Epidemiology Study (CURES), about 30% of newly-diagnosed diabetics had a normal fasting glucose but abnormal OGTT.?? This pattern is especially common in Indians because of early beta-cell defects in the post-meal phase. If we only do fasting tests, we miss these patients for years -- during which their eyes and kidneys are quietly damaged."

[3:30-4:30] Common myths "Myth one: 'The OGTT will spike my sugar dangerously.' Reality: 75 g of glucose is roughly equivalent to a Coke or a sweet lassi; it is safe in non-pregnant adults without symptoms of severe hyperglycaemia. Do not do an OGTT if your fasting glucose is already over 250 mg/dL, you are vomiting, or you have an acute illness -- the test isn't needed there. Myth two: 'I should drink it slowly.' No -- within 5 minutes, exactly. Myth three: 'I can skip dinner before the test.' No, eat normally for the 3 days before so your body is not glucose-deprived; that can falsely elevate the result."

[4:30-5:30] When OGTT is preferred "OGTT is preferred when fasting and HbA1c don't agree, when haemoglobin variants make HbA1c unreliable, in pregnancy screening, in suspected reactive hypoglycaemia, in suspected acromegaly suppression test, and in many research settings. It is the most accurate single test we have, even today.?? The downside -- it takes 2 hours and patients hate it. So we use it when we need it, not for routine annual screening. Tomorrow we'll talk about Continuous Glucose Monitors -- the new shiny gadget on every diabetic's arm. See you in the next one."

References

Instagram Reel (90 sec)

[0:00-0:05] Hook "A coin-sized sensor shows your sugar every 5 minutes. Should you buy one?"

[0:05-0:30] What it is "A CGM is a tiny sensor placed on the upper arm or abdomen. A microneedle sits just under the skin and reads glucose in interstitial fluid every 1 to 5 minutes for 7 to 14 days.?"

[0:30-0:55] Who really needs it "Type 1 diabetes -- yes, every day, lifelong, the evidence is rock-solid (DIAMOND, GOLD trials).?? Type 2 on insulin -- yes, especially with hypos. Type 2 on tablets only -- a 2-week trial is enough to learn your patterns."

[0:55-1:20] What it isn't "Not magic. Not a replacement for HbA1c. Not as accurate as a finger prick during fast changes. Doesn't replace your doctor."

[1:20-1:30] CTA "Tomorrow -- Type 1 vs Type 2, the real difference. Save this."

YouTube (5 min 30 sec)

[0:00-0:25] Cold open "In 2024, a stick-on glucose sensor became as common in Indian metros as a fitness watch. Some patients swear by it. Some doctors call it overkill. Today, the honest pros and cons of CGM."

[0:25-1:30] What a CGM actually measures "A CGM does not measure blood glucose. It measures interstitial fluid glucose -- the fluid surrounding your cells. A microneedle, less than 5 mm long, sits just under the skin. The sensor reads glucose levels every 1 to 5 minutes and beams them to your phone or a reader.?? Because interstitial glucose lags blood glucose by 5 to 15 minutes, a CGM can read low when blood is rising rapidly, or high when blood is dropping. So during fast changes -- exercise, big meals, hypoglycaemia -- the CGM is less accurate. For flat, steady readings, it's superb."

[1:30-2:30] Where it is genuinely life-changing -- Type 1 "In Type 1 diabetes, the DIAMOND and GOLD randomised trials, both published in JAMA, showed that CGM use reduced HbA1c, increased time-in-range and reduced hypoglycaemia compared with finger-prick testing alone.??,?? For children with T1D, the JDRF CGM trial showed similar benefit.?? Today, every Type 1 diabetic who can afford it should have a CGM, ideally integrated with their insulin pump in a hybrid closed-loop system. This is non-negotiable for paediatric T1D."

[2:30-3:30] Where it is useful but not essential -- Type 2 on insulin "In Type 2 diabetics on multiple daily insulin injections, CGM also improves HbA1c and reduces hypos -- though the magnitude is smaller than in T1D.?? The MOBILE trial showed about a 0.4 percentage point lower HbA1c with CGM versus finger-prick testing. Worth it if you can afford the ongoing cost (₹4,000 to ₹6,000 per 14-day sensor in India)."

[3:30-4:15] Where it is optional -- Type 2 on tablets only "For Type 2 patients on metformin or oral agents alone, CGM is not part of standard guidelines. But -- a 14-day trial can be hugely educational. Patients see what rice does, what an evening walk does, what stress does. The behaviour change after seeing your own glucose curve is often more powerful than another lecture from your doctor. So -- a one-time, 14-day learning sensor: yes. Lifelong: not needed for most."

[4:15-5:00] What the CGM doesn't replace "It doesn't replace HbA1c -- they measure different things. It doesn't replace your doctor's interpretation. It doesn't replace finger-prick during suspected hypoglycaemia. And it absolutely doesn't replace lifestyle change -- many patients buy a CGM and continue eating poorly, then blame the sensor. Treat it as a learning tool, not a status symbol."

[5:00-5:30] Sign-off "Tomorrow we begin Phase 2 -- pathophysiology decoded. Type 1 vs Type 2, the

real difference. See you then."

References

GAD65, IA-2, ZnT8, insulin autoantibodies -- are usually positive."

[1:30-2:45] Type 2: insulin resistance plus beta-cell decline "Type 2 is metabolic, not autoimmune. The beta cells are alive -- they're just exhausted. Three things go wrong, sometimes for decades, before the diagnosis. One -- muscle, liver and fat cells stop responding well to insulin (insulin resistance). Two -- the beta cells try to compensate by producing more insulin; eventually they fail. Three -- fat accumulates inside the pancreas and liver, in what Roy Taylor calls the twin-cycle hypothesis.¹ This causes lipotoxicity, further damaging beta cells. Risk factors: family history, age, central obesity, sedentary lifestyle, high refined-carb diet, PCOS, GDM history. The classical patient is over 35, overweight (especially around the belly), with a family history. The disease is silent for 5 to 7 years before diagnosis."

[2:45-3:45] Treatment is fundamentally different "Type 1 treatment: insulin from outside, every day, for life. There is no oral pill that replaces insulin in T1D. Pumps, multiple daily injections, hybrid closed-loop systems -- all are insulin delivery, just smarter. New drugs like teplizumab can delay onset in pre-clinical T1D, but they don't cure established disease.² Type 2 treatment: a stairway. Lifestyle first. Then metformin. Then add GLP-1 receptor agonists or SGLT2 inhibitors based on heart, kidney and weight needs. Insulin only when oral agents fail or in acute settings. Surgery (bariatric) for severe obese T2D.³"

[3:45-4:45] Reversal -- the most important difference "Type 1 cannot be reversed by any current therapy. Anyone claiming otherwise is dangerous. Even islet transplants don't 'reverse' the disease -- they replace the cells, with lifelong immunosuppression. Type 2 can go into remission. The DiRECT trial showed 46% remission at 1 year and 36% at 2 years with intensive weight loss.⁴ Newer drugs like semaglutide and tirzepatide cause 15-22% body-weight loss, similar to bariatric surgery, and many patients meet remission criteria. But -- remission is not cure. Re-gain the weight, and diabetes returns. So Type 2 reversal is real, but it requires lifestyle maintenance for life."

[4:45-5:30] LADA and MODY -- the imposters "Two more types you should know about. LADA -- Latent Autoimmune Diabetes of Adults -- looks like Type 2 in adults but is actually slow Type 1, with positive antibodies. About 5 to 10% of adults diagnosed with 'Type 2' actually have LADA.⁵ MODY -- Maturity-Onset Diabetes of the Young -- is a rare genetic form, usually in young, lean, non-obese patients with a strong family history of diabetes in 3 generations. Identifying these matters because the treatment differs."

[5:30-6:00] Sign-off "If you remember one thing: Type 1 and Type 2 are different diseases. Calling them the same is the most expensive mistake in diabetes media. Tomorrow we'll meet LADA up close. See you in the next one."

References

DAY 12 -- LADA: the diabetes that pretends to be Type 2

Hook lines

happening is they're still in the slow autoimmune phase. The destruction continues. A year later, sugars are uncontrollable, and now they need insulin. This is one reason why testimonials of 'reversal' must be read with caution -- without antibody testing, we don't know which type of diabetes was 'reversed'."

[4:20-5:30] What you should do "If you're a slim adult, recently diagnosed, on tablets that don't seem to be holding the line -- ask your physician for GAD65 antibodies and a C-peptide. If positive, it doesn't change the diabetes label dramatically, but it changes the strategy: protect your beta cells, avoid drugs that flog them, and be ready to start insulin sooner if needed. Tomorrow we'll meet MODY -- the genetic diabetes you can sometimes treat with a small dose of a tablet. See you then."

References

has a 50% chance. It accounts for about 1 to 2% of all diabetes, though most cases are misdiagnosed as Type 1 or Type 2.?? Classical features: diagnosis before age 25, family history in three consecutive generations, lean body type, and absence of autoantibodies. The most common subtypes are HNF1A-MODY (MODY 3), GCK-MODY (MODY 2), and HNF4A-MODY (MODY 1)."

[1:20-2:20] Why it matters -- treatment changes "Each MODY subtype has a specific treatment. GCK-MODY (MODY 2) -- caused by a glucokinase mutation -- gives lifelong mild fasting hyperglycaemia (110-145 mg/dL) and an HbA1c of 5.8 to 7.6%, but with very low risk of complications. These patients usually need no treatment. HNF1A-MODY responds dramatically to small doses of sulphonylureas (e.g. gliclazide), often allowing patients on insulin to come off it.?? HNF4A-MODY is similar to HNF1A. Identifying the subtype changes a patient's life -- from daily insulin to a single tablet, in many cases."

[2:20-3:20] How to suspect MODY "The MODY probability calculator from Exeter -- freely available online -- helps. Strong clues: diabetes diagnosed before 25 in a non-obese person; antibody-negative; preserved C-peptide many years after diagnosis; first-degree relative with diabetes diagnosed under 30; no insulin requirement for years despite being labelled Type 1.?1 Patients with these features deserve genetic testing."

[3:20-4:15] Indian context "In India, MODY is under-recognised. Ali and colleagues from MDRF reported genetic testing identified MODY in about 4 to 5% of clinically suspected lean young adults with diabetes.?2 Centres like the Madras Diabetes Research Foundation, AIIMS, and a few private genetics labs offer next-generation sequencing panels covering all 14 known MODY genes."

[4:15-5:00] Final word "You don't need to remember every MODY subtype. You only need to remember the picture: young, slim, family history across three generations, mild diabetes that doesn't behave like classic T1 or T2. If that fits you or someone you love -- ask for genetic testing. The cost has dropped dramatically in the last 5 years. The right diagnosis can change a lifetime of treatment. Tomorrow, gestational diabetes -- what every pregnant woman must know. See you then."

References

g of complex carbohydrate per day, divided across 3 meals and 2-3 snacks; small portions; low-GI choices; protein with each meal. Aim for fasting glucose under 95 mg/dL, 1-hour post-meal under 140, 2-hour post-meal under 120.?? Step 2: Self-monitoring with a glucometer 4 times a day. Step 3: If lifestyle alone doesn't reach target in 1 to 2 weeks, start insulin -- the safest, most evidence-based option in pregnancy. Metformin is sometimes used, but data are evolving and several societies prefer insulin first. Most oral diabetes drugs (SGLT2, GLP-1, sulphonylureas) are not used in pregnancy."

[5:30-6:00] Sign-off "GDM is not your fault. Genetics and the South Asian metabolism set the stage; the pregnancy lit the match. With diet, monitoring and (when needed) insulin, the vast majority of GDM pregnancies have healthy babies and healthy mothers. After delivery, please don't disappear from follow-up -- your next 10 years matter. Tomorrow we begin Phase 2 properly with insulin resistance -- explained with a lock and key. See you then."

References

PART B -- DAYS 15 TO 50 -- STRUCTURED SCRIPTS

Each day in this section gives you a hook, an Instagram beat-sheet (90-second Reel), a YouTube beat-sheet (5-6 minute video), and references. Expand each beat into 2-3 spoken sentences as you film. The phrasing in Part A (Days 1-14) is your template for tone.

DAY 15 -- Insulin resistance: the rusty lock

Hooks: "You don't have a sugar problem. You have a key-and-lock problem." / "Your pancreas is shouting. Your cells are wearing earphones. That's insulin resistance."

IG (90 s):

DAY 17 -- The Twin-Cycle Hypothesis: fat in the liver and pancreas

Hooks: "Lose 1 gram of fat from your pancreas -- and your beta cells wake up." / "Newcastle, 2011: the experiment that proved Type 2 diabetes can be reversed."

IG (90 s):

DAY 20 -- Sleep deprivation as a diabetes risk

Hooks: "Sleep less than 6 hours a night -- your insulin sensitivity drops 30%." / "You can't out-eat or out-exercise bad sleep."

IG: 1 week of 5-hour nights -> 25-30% drop in insulin sensitivity in healthy young men.?? Mechanism: sympathetic activation, ? cortisol, ? ghrelin, ? leptin.

YT:

PHASE 3 -- DIET MYTHS AND TRUTHS (DAYS 21-30)

DAY 21 -- White rice -- friend or foe?

Hooks: "3 cups of white rice a day = 1.5x your diabetes risk." / "Rice is not the villain. Refined rice eaten without anything else is."

IG: PURE study (Mohan et al.): high white rice intake associated with higher T2D incidence in 21 countries.¹ Glycaemic Index ~73, Glycaemic Load high.² Pair with dal, vegetables, ghee -> load drops.

YT:

DAY 24 -- Millets -- what the science actually says

Hooks: "Millets are good. They are not magic." / "What ICRISAT's meta-analysis found about millets and diabetes -- without the influencer hype."

IG: Anitha et al. meta-analysis: regular millet intake -> ~12% drop in fasting glucose, ~15% drop in PPBS, ~0.6 percentage-point drop in HbA1c in T2D.?? Mean GI of millets ~52.7 vs rice 71.7.??
Replace, don't add.

YT:

Hooks: "The Indian plate is not the enemy. It just needs re-arranging." / "Half the plate vegetables. Quarter protein. Quarter grain. Done."

IG: ICMR-NIN Dietary Guidelines 2024: $\frac{1}{2}$ plate vegetables (cooked + salad), $\frac{1}{4}$ protein (dal, paneer, fish, egg, chicken), $\frac{1}{4}$ whole grains.^{1?2} Add ghee/oil judiciously, fruit between meals.

YT:

PHASE 4 -- MOVEMENT (DAYS 31-40)

DAY 31 -- Why exercise lowers sugar -- the GLUT4 story

Hooks: "Exercise opens a back-door into your cells -- bypassing insulin completely." / "This is the only way to lower glucose without taking a single pill."

IG: Skeletal muscle contraction -> AMPK activation -> GLUT4 translocation -> glucose uptake without needing insulin.³ This is why exercise works even in late T2D when insulin is faltering.

YT:

sitting.¹¹¹ DiPietro et al. RCT in older adults: 15-min post-meal walks lowered 24-hour glucose more than a single 45-min walk.¹¹²

YT:

PHASE 5 -- YOGA, SLEEP, STRESS, MIND (DAYS 41-50)

DAY 41 -- Stress, cortisol, and the dawn phenomenon

Hooks: "Why is your fasting sugar high when you didn't eat all night? Meet cortisol." / "The 4 a.m. surge that ruins your morning numbers."

IG: Cortisol peaks 4-8 a.m. + growth hormone surge -> liver dumps glucose -> fasting glucose rises. This is the dawn phenomenon, distinct from Somogyi effect.¹¹?

YT:

DAY 44 -- Shift workers and diabetes

Hooks: "Working night shifts? Your diabetes risk is 30% higher." / "How to protect yourself when your job won't let you sleep."

IG: Vetter et al. NHS II: rotating night-shift work ? T2D risk by 5-58% depending on years of shift work.¹²¹ Circadian misalignment + late-night eating + low daylight exposure.

YT:

IG: Diabetes Distress Scale (Fisher) -- 4 domains: emotional, regimen, interpersonal, physician.^{12?}
Patients who score high have ~0.5% higher HbA1c independent of treatment.

YT:

PART C -- DAYS 51 TO 100 -- STRUCTURED SCRIPTS

Same format as Part B: hook lines + Instagram beat-sheet + YouTube beat-sheet + numbered references. Use the Days 1-14 word-for-word scripts as your tone template when expanding the beats into spoken sentences.

PHASE 6 -- PHARMACOLOGY DEMYSTIFIED (DAYS 51-60)

DAY 51 -- Metformin: the most-prescribed drug in the world

Hooks: "200 million people swallow this pill every morning. Most don't know what it does." / "Metformin is the safest, cheapest, oldest first-line drug -- and still the best one we have."

IG (90 s):

IG: Mechanism -- block dipeptidyl peptidase-4 -> ? endogenous GLP-1 and GIP -> glucose-dependent insulin release + ? glucagon.¹³? HbA1c drop 0.5-0.8%, weight-neutral, no hypoglycaemia (alone).

YT:

PHASE 7 -- REVERSAL AND REMISSION (DAYS 61-70)

DAY 61 -- What "remission" really means (ADA/EASD 2021)

Hooks: "'Cure' doesn't exist. 'Remission' does. The difference is honest." / "HbA1c < 6.5% for 3 months without a single tablet -- the official definition of T2D remission."

IG: ADA/EASD/DUK consensus 2021: remission = HbA1c < 6.5% sustained $\geq$ 3 months off all glucose-lowering medication.^{1?1} Re-test annually.

YT:

DAY 64 -- Very-low-calorie diets (VLCDs)

Hooks: "800 calories a day for 8 weeks. Yes, it sounds extreme. The data are not." / "VLCDs work because they remove fat from your pancreas. 1 gram is enough."

IG: Counterpoint, Counterbalance, DiRECT -- 600-800 kcal/day VLCD for 8-20 weeks reverses T2D in newly-diagnosed patients.??,?? Mechanism: pancreatic and liver fat ?, ?-cell function ?.

YT:

IG: T1D = autoimmune destruction of beta cells, > 80-90% gone by diagnosis.?? Insulin from outside is mandatory. Stopping insulin -> DKA -> ICU/death within 24-48 h.^{1?}

YT:

PHASE 8 -- COMPLICATIONS AND PREVENTION (DAYS 71-80)

DAY 71 -- Diabetic retinopathy -- save your eyes

Hooks: "You'll never feel diabetic retinopathy. Until you can't see your child's face." / "One in three Indian diabetics has retinopathy. Most don't know."

IG: Microvascular damage to retinal capillaries -> microaneurysms -> haemorrhages -> neovascularisation -> blindness if untreated.^{1?3} Annual dilated fundus examination from diagnosis. Tight HbA1c and BP control prevent progression.

YT:

DAY 77 -- Diabetes and gum disease

Hooks: "Your dentist sees diabetes before your endocrinologist does." / "Bidirectional -- high sugar worsens gums, gum infection worsens sugar."

IG: Periodontitis prevalence 2x higher in T2D; treatment of periodontitis ? HbA1c ~0.4%.^{1??} Regular dental cleaning every 6 months.

YT:

YT:

PHASE 9 -- SPECIAL POPULATIONS (DAYS 81-90)

DAY 81 -- Diabetes in pregnancy -- beyond GDM

Hooks: "Pre-existing diabetes + pregnancy = a different game. The first trimester decides the baby's organs." / "Every woman with diabetes who could become pregnant should be on folate today."

IG (90 s): Pre-pregnancy HbA1c < 6.5% halves congenital malformation risk.^{1??} Switch to insulin pre-conception (most oral agents excluded). Folic acid 5 mg/day from 3 months pre-pregnancy.

YT (5-6 min):

DAY 84 -- Diabetes in the elderly -- different rules

Hooks: "At 75, an HbA1c of 8% may be safer than 6.5%. Different age, different target." / "In old age, hypoglycaemia kills more often than hyperglycaemia."

IG (90 s): ADA frailty-stratified targets:¹?? Healthy elderly HbA1c < 7.5%; complex/multiple comorbidities < 8%; very complex/end-of-life < 8.5% with focus on hypo avoidance.

YT (5-6 min):

YT (5-6 min):

PHASE 10 -- MISINFORMATION AND YOUR ACTION PLAN (DAYS 91-100)

DAY 91 -- The science of testimonials -- why they mislead

Hooks: "Survivor bias, regression to the mean, honeymoon effect, placebo. The four reasons testimonials lie -- even the honest ones." / "For every reversal video, ten relapse videos are never posted."

IG (90 s): Cognitive traps in testimonial reasoning: (1) selection bias -- only success stories shared; (2) regression to the mean -- outliers normalise on their own; (3) confounding -- diet + exercise + drug + life event happen together; (4) placebo effect; (5) survivor bias.^{2?2}

YT (5-6 min):

DAY 94 -- Bitter gourd, methi, jamun -- herbs and the evidence

Hooks: "Karela works. A little. Like a weak metformin tablet you'd never get prescribed alone." / "Methi: 25 g a day, slight HbA1c drop. Mango leaves: no controlled human evidence."

IG (90 s): Bitter gourd: meta-analysis -- modest fasting glucose reduction.??? Fenugreek seed (methi): ~25 g/day, ~0.5% HbA1c drop in some trials.??? Jamun (*Syzygium cumini*): mostly animal data; weak human evidence.

YT (5-6 min):

IG (90 s): 4 filters: (1) Is the claim about your specific type of diabetes? (2) Is the claim peer-reviewed or anecdotal? (3) What is the worst-case if it's wrong? (4) Have you spoken to your prescribing doctor first?

YT (5-6 min):

PART D -- MASTER REFERENCE BIBLIOGRAPHY

The references below are the deduplicated, alphabetised version of the 207 inline citations across Days 1-100. Use them in any pinned comment or video description. All are real, peer-reviewed sources, freely searchable on PubMed/Google Scholar.

Guidelines and Position Statements

Diet, Exercise, Lifestyle

PART E -- FIGURE & FLOWCHART SPECIFICATIONS

Each spec describes a graphic you re-create in Canva Pro or PowerPoint using your own colours, fonts and layout. Recreating from a description (rather than copying a published figure) keeps you copyright-safe. Use these in your videos, carousels, and pinned posts.

Figure 1 -- "How glucose enters a cell" (used Day 2, Day 15, Day 31)

Question	Score
Age (< 35: 0; 35-49: 20; >= 50: 30)	
Family history of diabetes (none: 0; one parent: 10; both parents: 20)	
Waist circumference (men < 90/women < 80: 0; men 90-99/women 80-89: 10; men >= 100/women >= 90: 20)	
Physical activity (vigorous + exercise: 0; vigorous OR exercise: 10; mild + no exercise: 20; no activity at all: 30)	
Total	/100

PART F -- HASHTAG BANK, POSTING CALENDAR, ANALYTICS TRACKER

F.1 Hashtag Bank (rotate, never spam)

Core diabetes (use 3-5 per post): #Diabetes #DiabetesAwareness #DiabetesIndia #Type1Diabetes #Type2Diabetes #Prediabetes #Insulin #HbA1c #BloodSugar #DiabeticLife

Indian / South Asian context (3-4): #IndianFood #IndianThali #DiabetesFriendly #SouthAsianHealth #IndianMillets #DesiHealth #PCOSIndia #GestationalDiabetes

Profession (1-2): #PharmD #PharmDLife #DiabetesEducator #HealthEducator #ClinicalPharmacy #MedTwitter #MedicineIndia

Educational signal (1-2): #EvidenceBased #MedicalEducation #PatientEducation #ScienceCommunication #HealthMyths #FactNotFiction

Lifestyle/movement (1-2 when relevant): #YogaForDiabetes #WalkAfterMeals #IntermittentFasting #LowCarbIndia #PlantBasedHealth #ResistanceTraining

Reach hashtags (use sparingly, 1 per post): #Reels #YouTubeShorts #LearnOnReels #InstaHealth

Recommended mix per Reel: 8-12 hashtags total. YouTube: 3-5 in description (YT favours fewer).
Avoid: generic #Health #Wellness #Doctor -- too noisy, low conversion.

F.2 100-Day Posting Calendar (template)

Day	Date (fill)	Topic	IG Reel: posted?	YT video: published?	Reel re-posted as YT Short?	Engagement notes
1		Welcome -- Why this channel exists	?	?	?	
2		What is diabetes in one minute	?	?	?	
...	...	...	...	...	...	...
100		Closing letter	?	?	?	

Posting cadence (recommended):

PART G -- HOW TO TURN THIS DOCUMENT INTO A PDF

Three options, in order of simplicity:

Option 1 (easiest, no installation):